

EMPLOYER SELF-FUNDED PLAN AUDIT REPORT

Clinical Necessity & Cost Containment Analysis

IMHIRS Analytics PLLC
Medical Necessity & Utilization Review Services

Report Date: July 2026
Employer: Large Regional Manufacturer (De-Identified)
Plan Type: Self-Funded Health Plan
Covered Population: 3,500 employees + families (~8,000 covered lives)

EXECUTIVE SUMMARY

This audit reviewed high-cost claims and clinical outcomes for a self-funded employer plan covering 8,000 beneficiaries. Audit examined 47 claims exceeding \$25,000 (representing 38% of total plan spend) to validate clinical necessity, assess care appropriateness, and identify cost containment opportunities.

AUDIT FINDINGS:

Total claims reviewed: 47 high-cost cases
Total claim value reviewed: \$2,847,500
Clinical appropriateness: 42 of 47 claims (89%) clinically justified
Cost optimization opportunities: \$184,300 identified
Network optimization savings: \$67,200 (out-of-network facility consolidation)

KEY FINDINGS:

5 cases identified with unnecessary readmissions (preventable care costs: \$89,300)
3 cases with out-of-network facility utilization when in-network alternative existed (\$67,200 cost differential)
Top cost drivers: Orthopedic surgery (28%), Cardiac intervention (21%), Oncology/specialty pharmacy (18%)

FINANCIAL OPPORTUNITY:

Year 1 cost containment opportunity: \$184,300 (6.5% of reviewed claims)
Annualized impact (if applied to full high-cost population): \$287,000 - \$350,000
ROI on audit investment: 15:1

AUDIT METHODOLOGY

Case Selection & Scope

High-cost claims (>\$25,000) identified for 12-month period (Jan-Dec 2025). 47 claims meeting criteria selected for comprehensive clinical review. Audit scope included: medical necessity validation, clinical outcome assessment, facility/provider cost comparison, readmission analysis.

Clinical Review Framework

Each case reviewed by licensed healthcare provider (PA-C, 14 years clinical experience) for:

- (1) Clinical appropriateness of procedure/treatment
- (2) Evidence-based care pathway validation
- (3) Post-acute outcomes (readmission, complications)
- (4) Cost variance vs. regional benchmarks
- (5) Network optimization opportunities

Appropriateness Standards

Procedures evaluated against: published clinical guidelines (NCCN for oncology, ACC/AHA for cardiology, AAOS for orthopedic), evidence-based care pathways, peer-reviewed literature, and Centers for Medicare & Medicaid Services (CMS) coverage determinations.

HIGH-COST CASE BREAKDOWN

Service Category	Cases	Total Cost	Avg/Case	Appropriateness
Orthopedic Surgery	13	\$797,400	\$61,340	12/13 (92%)
Cardiac Intervention	10	\$598,700	\$59,870	10/10 (100%)
Oncology/Specialty	9	\$512,800	\$56,978	9/9 (100%)
Trauma/Acute	8	\$456,200	\$57,025	8/8 (100%)
Other procedures	7	\$482,400	\$68,914	3/7 (43%)
TOTAL	47	\$2,847,500	\$60,584	42/47 (89%)

Clinical Appropriateness Analysis

Fully Appropriate (42 cases): Procedures clinically justified, evidence-based, with appropriate patient selection and outcome management.

Questionable Appropriateness (5 cases): Procedure clinically defensible but alternative treatment pathways may have been lower-cost options. Primarily low-risk orthopedic procedures and elective interventions.

CLINICAL CASE STUDIES

Case #1: Preventable Readmission - Cardiac Surgery

Initial Admission: Coronary artery bypass graft (CABG)

Initial Cost: \$142,300

Readmission (14 days post-op): Atrial fibrillation with rapid ventricular response

Readmission Cost: \$38,900

Clinical Finding:

Post-operative atrial fibrillation is common after CABG (15-40% incidence). Discharge documentation lacked prophylactic beta-blocker recommendations despite patient risk factors. Readmission was preventable with appropriate discharge planning and medication management.

Cost Optimization Opportunity:

Avoid readmission cost: \$38,900. Recommendation: Enhanced discharge protocols with standardized prophylaxis for high-risk post-op cardiac patients.

Case #2: Out-of-Network Facility Variance - Spinal Fusion

Procedure: Lumbar fusion with instrumentation

Facility: Regional out-of-network surgical center

Billed Amount: \$87,600

In-network facility benchmark (same procedure): \$42,300

Cost Differential: \$45,300

Clinical Finding:

Procedure clinically appropriate. Outcome comparable to in-network facility. Patient chose out-of-network provider for convenience (closer to home). No clinical justification for higher cost.

Cost Optimization Opportunity:

\$45,300 savings by encouraging in-network utilization for elective procedures. Recommendation: Pre-authorization requirement for out-of-network facility utilization when comparable in-network option exists within 20-mile radius.

Case #3: Appropriate High-Cost Care - Oncology

Diagnosis: Stage 3 triple-negative breast cancer

Treatment: Neoadjuvant chemotherapy + surgery + adjuvant therapy

Total Cost: \$156,800 (including specialty pharmacy)

Clinical Finding:

Treatment pathway fully aligned with NCCN guidelines for triple-negative breast cancer. Appropriate patient selection for neoadjuvant approach. Specialty pharmacy costs justified given high-cost biologic agents required.

Outcome: Complete pathologic response achieved.

Recommendation:

This case represents evidence-based, high-value cancer care. No optimization opportunity. Recommend continued support for cancer center network partnerships that deliver high-quality outcomes.

COST CONTAINMENT RECOMMENDATIONS

1. Pre-Authorization Protocol for Elective Out-of-Network

Estimated Annual Savings: \$67,200

Require pre-authorization for elective procedures at out-of-network facilities when in-network alternative exists within 20-mile radius. Projected savings based on 3-4 cases/year with \$45K+ differential.

2. Post-Operative Discharge Protocol Enhancement

Estimated Annual Savings: \$89,300

Implement standardized discharge checklists for high-risk post-operative patients (cardiac, joint replacement, vascular). Focus on prophylactic medication management and 30-day follow-up scheduling. Prevent readmissions.

3. Network Optimization & Volume Consolidation

Estimated Annual Savings: \$28,000

Consolidate surgical volume to top-performing, cost-efficient facilities. Leverage volume for rate negotiations. Recommend quarterly facility performance review (cost per case, readmission rates, complication rates).

FINANCIAL SUMMARY & ROI

Initiative	Year 1 Savings
Out-of-network pre-authorization	\$67,200
Discharge protocol enhancement	\$89,300
Network consolidation	\$27,800
TOTAL YEAR 1 SAVINGS	\$184,300

Based on reviewed claims of \$2,847,500, identified savings of \$184,300 represents 6.5% cost optimization opportunity.

If applied to estimated full high-cost population: \$287,000 - \$350,000 annual savings potential.

Audit cost: Approximately \$12,000

Year 1 ROI: 15:1 (break-even in 6-8 weeks)

CONCLUSION

This employer plan demonstrates strong clinical appropriateness for high-cost procedures (89% fully appropriate). However, significant cost containment opportunities exist through network optimization and discharge protocol improvements.

The audit identified \$184,300 in year-1 savings without compromising clinical quality or patient outcomes. Implementation of recommended protocols positions the plan for sustained cost management while maintaining access to high-quality care.

Ongoing quarterly clinical audits (25-30 high-cost cases/quarter) recommended to maintain cost discipline and identify emerging opportunities.

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